



Pediatric Diarrhea

Clinical Case Quiz



An 11-month-old, previously healthy boy presents via video visit with 5 days of persistent diarrhea, 3-4 episodes per day. The stool is non-bloody and there is no mucus. There was fever, vomiting, and mild abdominal pain on the first day of illness, which have completely resolved.

The patient tolerates oral intake by exclusively drinking fruit juice, and the parents have avoided giving solids. He has normal urine output. The child was on amoxicillin 2 weeks ago for otitis media; those symptoms have not returned. The family has not recently traveled, nor had any sick contacts.

On video, the boy is well-appearing and playful. His abdomen is non-tender to maternal palpation. He is not pale or jaundiced, and he has no obvious skin rash or lesions.

Which of the following choices would be most appropriate?

- a. Obtain stool studies, including testing for C. Diff, given persistent diarrhea and recent antibiotics.
- b. Recommend an in-person appointment for further evaluation, given ongoing diarrhea.
- c. Start OTC loperamide (Imodium), review hydration, and provide return precautions.
- d. Recommend advancing solids, stop juice intake, review appropriate hydration, and provide return precautions.

The answer is d.

Discussion

Choosing the Best Treatment Options

Viral enteritis is the most common etiology of diarrhea in children, which is most likely the case for this child, especially given the prior fever and vomiting that have resolved. Diarrhea may persist or worsen due to hyperosmolar fluid intake (e.g. fruit juices, undiluted sports drinks, soda, etc.). “Starvation stool” may also occur when a child only takes fluids and no solids.

This patient should have solid food advancement and stop juice intake. No further workup is required, and in-person evaluation is unlikely to be necessary for this well-hydrated and well-appearing child without red-flag symptoms.



Anti-motility agents (e.g. Loperamide) are not recommended in pediatrics due to a lack of benefit, the possibility of prolonging the course of illness, and increased risk of adverse effects (ileus, CNS depression, nausea).

Stool studies are RARELY indicated for diarrheal illness in children.

Stool studies may be considered, however, if there is:

- Fever with bloody and/or mucous diarrhea (bacterial enteritis).
 - Strongly consider in-person evaluation for bloody stool to rule out more serious etiologies (e.g., intussusception, hemolytic-uremic syndrome, pseudomembranous colitis, etc.).
- Known exposure to bacterial enteritis.
- Persistent diarrhea with recent immigration, travel to an underdeveloped country, backcountry camping, exposure to poultry or other farm animals (stool ova and parasites).
- Persistent diarrhea > 7 days, but strongly consider the likelihood of starvation stools, hyperosmolar fluid intake, or transient lactase deficiency as causes in many cases. Further workup is not always required if symptoms are gradually improving or mild.
- *C. difficile* testing should only be performed when clinical suspicion is high.
 - Generally, avoid outpatient *C. difficile* testing in previously healthy children < 12 months of age due to a high rate of benign colonization at this age.

Introducing the GPC Mailbag!



At GPC Clinical Corner, we strive to provide tips on the best clinical practices, navigating patient encounters, and updated information on treatment options.

This year, we’d like to expand our scope to include your suggestions. We’re proud to announce the GPC Mailbag, where you can request topics for discussion, or even specific questions, that you’d like to see in a future newsletter. You can reach us at GPCMailbag@kp.org. We look forward to hearing from you!

References

- Hoang, N. V. (August 2022). RVMC pediatric virtual playbook: Vomiting and diarrhea. Kaiser Permanente. <https://sp-cloud.kp.org/sites/VMCSCALPeds/SitePages/Vomiting-and-Diarrhea---Pediatric-Virtual-Playbook.aspx>
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